

SECOND AMENDED STATEMENT OF FACTS AND CONTENTIONS (FORM 9A)

WC/2024/227 – Cory Lea Shepherd (Appellant) v Workers' Compensation Regulator (Respondent). Section 550(4), Workers' Compensation and Rehabilitation Act 2003.

Particulars are given by reference to the paragraph numbers of the Appellant's notice to admit facts served on the Respondent on 28 August 2026, to which the Respondent responded on 8 September 2026.

PART A

Appellant: Cory Lea Shepherd. Employer: State of Queensland (Queensland Health) – Logan Hospital Switchboard. Role: Administration Officer, Switchboard Services, classification AO3, a continuous shift working role.

Injury: major depressive disorder with anxious distress (DSM-5 296.23), diagnosed by the Appellant's treating psychiatrist on 24 October 2024.

The injury is a diagnosed medical condition arising from the Appellant's employment over the course of conduct pleaded at Part B.2 below, between approximately June 2023 and June 2024. It is not pleaded as arising from an event occurring on a single day. The date of injury recorded on the work capacity certificates of the treating general practitioner is 18 June 2024. That date is the practitioner's and is a matter for the medical evidence; the Appellant asserts no date of onset of his own.

Particulars: paragraphs 1 to 4, 13 to 16 of the notice to admit facts served 28 August 2026, admitted 8 September 2026.

PART B — MATERIAL FACTS

The documentary facts identified below have been admitted to the extent recorded in the Respondent's response of 8 September 2026. Those admissions provide evidence from which the Commission is invited to determine the nature, reasonableness and causal significance of the relevant circumstances.

1. Baseline — the medical record

1.1 The Switchboard receives emergency response notifications and distributes them to the response groups by category of emergency, and maintains the contact and paging information by which that is done.

Duty engaged: paragraphs 5, 7 to 8, 10 of that notice (the role description).

1.2 **Prior health.** Before 18 June 2024 the Appellant had not been diagnosed with or treated for depression. The general-practice record of 16 November 2023 (Dr Nanayakkara) records poor sleep with shift work, that the Appellant could not do shifts without a good sleep, "no psychological illness such as depression or psychosis", and mood good, with melatonin and temazepam prescribed that day. **[Tab M1 — general-practice records, Our Medical Ashmore, 1 January 2023 to 1 July 2024, obtained by the Respondent under the Form 29 signed 4 July 2025; Respondent's item 11.]** Relied upon as the contemporaneous record that shift work was affecting the Appellant's sleep, and of what was prescribed for it, before any claim or proceeding. Its clinical significance is a matter for the treating doctors. Private entries unrelated to the injury are redacted on the extracted pages and marked as such.

Particulars: paragraphs 2, 13, 214, 218, 263 to 265, 269 to 271 of the notice to admit facts served 28 August 2026, admitted 8 September 2026.

1.3 **The referral of 16 May 2024.** The referral letter of 16 May 2024 (Dr Zhao) renews a referral to a psychiatrist, Dr Amini, for "ongoing care and management". It lists the past medical history as the history-list items "26/10/2022 ADHD" and "26/10/2022 Anxiety", and lists the medications then current, **which include no antidepressant, no anxiolytic and no other psychotropic medication.** **[Tab M1.]** It was written the day after the email of 15 May 2024 at 6:23 pm asking the Appellant to retract his email of that afternoon. That sequence is pleaded as to date only; no connection between the two is alleged. No appointment with the psychiatrist was obtained on that referral.

Particulars: paragraphs 76 to 77 of the notice to admit facts served 28 August 2026, admitted 8 September 2026.

1.4 **Deterioration and first presentation.** The Appellant last worked a shift on 3 June 2024. On days in May 2024 and in the first week of June 2024 he drove to work, was unable to enter the building, and reported himself unfit from the car park. The date of injury recorded on the certificates is 18 June 2024. On 28 June 2024 (Dr Slawinski) the record shows "stress at work" and "upset by people not following rules", reason for visit anxiety. On 1 July 2024 (Dr Hawes) the record shows "work stress", "been there 5 years", that "they withhold pay at times, no overtime- not processed, manipulate his roster- so he works later than earlier", and "causing anxiety". **[Tab M1.]** The events those records name are the matters admitted on 8 September 2026 and pleaded at Part B.2 below. The records are relied upon as the contemporaneous record of what was reported, before any claim decision, dismissal or proceeding; not as a finding of fact as to mechanism.

1.5 **Certification.** The work capacity certificates of Dr Hawes dated 1 July, 11 August and 8 September 2024, and of Dr Ki Pang dated 7 August 2024, state the date of injury as 18 June 2024, that the Appellant was first seen for this injury on 1 July 2024, and certify no functional capacity continuously from 1 July to 6 October 2024. The referral to a psychiatrist is recorded on the certificate of 8 September 2024. Review Decision 69983 records at page 17 that the certificate of 1 July 2024 indicated "there was no pre-existing factor or condition", and that "This was maintained in all later work capacity certificates". **[Tab M2; Respondent's items 7 and 8.]** The medication box is unticked on each certificate.

1.6 **Diagnosis, 24 October 2024.** At the first consultation, on the referral, the treating psychiatrist told the Appellant, and confirmed in writing at 11:45 am that day, that he was "suffering from psychological injury of Major Depressive Disorder with anxiety state"; that fluoxetine was increased to two capsules "from today" and Seroquel 25 mg commenced at night, "to restore basic needs- sleep, eating and routine". The Appellant's email of 5:12 pm that day reads "I have finally been able to see a psychiatrist today". **[Tab M3; Respondent's item 9.]** Read with 1.5, it records the progression from the injury certified as anxiety and stress on 1 July 2024 to a depressive disorder by 24 October 2024.

1.7 **The report of 13 February 2025.** Diagnosis: Major Depressive Disorder with anxious distress (DSM-5 296.23). Severity, functional effect and treatment (fluoxetine increased to three capsules daily; quetiapine 25 mg at night). The treating clinician records the origin as "workplace stress stemming from issues with management and rostering"; that the issues "began approximately one year ago when a new manager was appointed"; that pay was "withheld or delayed for up to five months at a time, leading to significant financial stress"; and that "premature exposure to the workplace is more likely result in significant deterioration". **[Tab M4; Respondent's item 10.]** The stressors so recorded correspond to matters admitted on 8 September 2026: management and rostering, and the night-shift line; the shorter break; and pay withheld or delayed from the 5 February 2024 fortnight, uncorrected at

13 May, "claims older than 3 months" on 28 May, a claim effective 30 March recorded "Part Completed" on 30 May, and the two February claims absent from the myHR report.

Particulars: paragraphs 193, 196 to 197, 203, 210 to 212, 220, 258 to 260 of the notice to admit facts served 28 August 2026, admitted 8 September 2026.

1.8 The report's footer, and the author's position. The report bears a footer reading "for the only reason of clinical information and not for medico-legal use". The author's emails of 5 and 8 September 2026 state of his records "You can use them according to the need to support your legal issues", and identify the report of 13 February 2025 as "the report that captures the relevant information you have requested" on the matters in issue. **[Tab M5.]** The report is not relied upon for attribution among the individual events pleaded at Part B.2, nor for any matter after 24 October 2024 as a cause of the injury.

1.9 Current capacity. The Employee Capability Checklist completed by Dr Day Hong Ma on 3 July 2026 records current capacity and restrictions and the continuing effect of the injury, including "symptom exacerbation on exposure to the identified workplace stressors". **[Tab M6.]** Relied upon for effect and capacity only, not for causation; it post-dates 1 July 2024.

1.10 Outstanding, and the finding in the decision under appeal. The clinical records of the treating psychiatrist from 24 October 2024 were offered by the practice on 5 September 2026 and have been requested; they are not yet received and will be served on receipt. On the medical evidence then before it, the Respondent's review of 24 October 2024 found that the Appellant "sustained a personal injury of a psychological nature" and stated: "Having regard to the medical evidence, I am satisfied your employment was a significant contributing factor to the psychological injury". The claim was rejected under section 32(5), not section 32(1). **[Tabs M7 and M9.]** The hearing is de novo and that finding does not bind the Commission; it is relied upon as an admitted document.

Particulars: paragraphs 261 to 262 of the notice to admit facts served 28 August 2026, admitted 8 September 2026.

2. Causative stressors (composite course, June 2023 – June 2024)

STRESSOR 1 — THE CONDITIONS IN WHICH THE WORK WAS CARRIED ON

S1.1 The duty to keep the contact information accurate. A key responsibility of the position is to collate information and maintain the Omnivista database and SharePoint so that the information held within Switchboard Services is accurate and appropriate, and to maintain the database and registers of paging and contact information with accurate and current information. That duty is the duty by reference to which each of S1.2 to S1.8 below is pleaded.

Duty engaged: paragraphs 6 to 7 of that notice (the role description).

S1.2 The removal of the means of performing that duty, 18 July 2023. On 18 July 2023 the Switchboard Telecommunications Coordinator notified the Appellant and the Switchboard team that she would be removing everyone's access to the database, and that she had removed the Contact and Number Changes book from the room. The Respondent's own review decision records the same matters. The Respondent does not allege that access to the database was restored to the Appellant at any time before 18 June 2024. The duty at S1.1 remained on the position throughout that period.

Particulars: paragraphs 39 to 40, 45 to 48, 55 of the notice to admit facts served 28 August 2026, admitted 8 September 2026.

S1.3 The repair path, and its availability. From 18 July 2023 an operator who identified an incorrect or missing entry could not correct it. An amendment was to be sought directly from the Coordinator, whose stated working hours were every Tuesday and every second Monday between 8:00 and 16:00 in a temporary project role, or, if she was not present and the matter was urgent, from the Switchboard Manager. A request made after hours, overnight, on a weekend or on a public holiday was to wait until one of them returned. The Appellant pleads no characterisation of any person; the availability of those two persons is pleaded because it determined how long an incorrect entry remained uncorrected.

Particulars: paragraphs 41 to 44 of the notice to admit facts served 28 August 2026, admitted 8 September 2026.

S1.4 The criticality of the function, as admitted by the Respondent. On 18 February 2026 the Respondent admitted that "Maintaining accurate contact details for medical staff is a critical function of the Switchboard to ensure effective clinical handover and patient safety", adding that there was a procedure in place for this to occur, and denied a further paragraph of that notice on the ground that there was already a procedure in place. The procedure in place from 18 July 2023 is the procedure pleaded at S1.3 above. Its operation between 2 and 20 May 2024 is pleaded at S1.7 and S1.8 below.

Particulars: paragraphs 283, 289 to 291 of the notice to admit facts served 28 August 2026, admitted 8 September 2026.

S1.5 The further changes to process, April and May 2024. On 15 April 2024 the Switchboard Manager notified Logan Switch, copied to the Director and the Switchboard staff, that she and the Coordinator had added themselves to an after-hours on-call arrangement, that the process during office hours remained the same and that staff were to contact her through the switchboard, the office or her mobile, and that the new process was effective from that day. On 19 April 2024, as recorded in the review decision, she notified the team that due to errors being made with respect to data entry a new process was to be followed which included more checks to ensure accuracy of data entry. On 13 May 2024 the on-call roster for the following fortnight was circulated. On 14 May 2024 the process for notifying unavailability for a rostered shift was also directed through the same person.

Particulars: paragraphs 49 to 54, 111 to 113, 162 to 166 of the notice to admit facts served 28 August 2026, admitted 8 September 2026.

S1.6 The obligation to consult, and the absence of any record of consultation. Clause 3.2 of the Queensland Public Health Sector Certified Agreement (No. 11) 2022 provides that the requirement of consultation is never to be treated perfunctorily or as a mere formality, that consultation involves more than a mere exchange of information and requires participants to contribute to the decision-making process "not only in appearance, but in fact", and that the process requires the exchange of timely information so that the parties have "an actual and genuine opportunity to influence the outcome, before a final decision is made". Clause 4.1.4 requires a business case to be tabled for consultation for significant organisational change including major alterations to current service delivery arrangements. The change notified on 15 April 2024 was stated to be effective the day it was notified, and the Respondent's amended List of Documents records no document of any consultation with Switchboard operators before it. The Respondent does not allege that agreement under clause 6.2 of the Award was obtained, or that a ballot of affected employees was conducted, before that change. The employer conducted a consultation process on other occasions, including in November and December 2024, under which agreement is defined as the consent of a majority of affected employees. On 20 May 2024 the Switchboard Manager wrote that updated procedures could be put to the team for consultation before implementing.

Duty engaged: paragraphs 11 of that notice (the role description).

Particulars: paragraphs 101, 167 to 170, 176 to 177, 180 to 181, 273 of the notice to admit facts served 28 August 2026, admitted 8 September 2026.

S1.7 What arrived at the console: the misdirected emergency calls, 2 to 9 May 2024. A key responsibility of the position is to participate in the Emergency Response process by receiving emergency response notifications and distributing them to the

appropriate response groups, dependent on the category of emergency, as per emergency code procedures, strictly adhering to protocols and timeframes; to maintain call queues to a minimum at all times; and to operate under pressure where high volume call traffic is concerned. On 3 May 2024 at 3:06 pm the MASPER Registrar wrote to the Switchboard Manager listing five occasions on 2 and 3 May on which calls had reached the wrong destination, including an occasion at 14:46 recorded as "MET call team called x5290 asking where MET call was located 'VHUB' - switchboard could not tell them where VHUB was. Had to be redirected by MASPER". On 8 May at 5:28 pm she wrote again listing four further occasions. Those nine occasions fell while the arrangement notified on 15 April 2024 was in force. The first reply was sent on 9 May at 9:20 am, five days, eighteen hours and fourteen minutes after the first email, and asked the reporting clinician to confirm her business hours because they were not provided on the rosters, noting that her handset was switched off. At 10:15 am the same day the Switchboard Manager wrote to Logan Switch introducing a new process and referring to "many ongoing issues raised by the MASPER and the medical department about calls being transferred to the wrong medical teams". The Respondent does not allege that any communication was sent to Logan Switch or to the Switchboard staff concerning those occasions at any time after 3 May 2024 and before 10:15 am on 9 May 2024.

Duty engaged: paragraphs 5, 8, 10 of that notice (the role description).

Particulars: paragraphs 56 to 69 of the notice to admit facts served 28 August 2026, admitted 8 September 2026.

S1.8 What arrived at the console: the Integrated Respiratory Service, 15 to 20 May 2024. On 15 May 2024 at 11:47 am an Administration Officer of the Integrated Respiratory Service wrote to Logan Switch, marked of High importance, asking that the number registry or directory be amended to show that an extension belonged to that Service, and stating that it was not Respiratory Medical Outpatients and had no doctors working out of that area. No response is alleged before 20 May. On 20 May at 11:03 am she wrote again, marked of High importance, stating that calls continued to be put through and that "we can not help patients or other clinical staff with OPD issues". At 2:05 pm that day, from the Logan Switch account and within the office hours the Switchboard Manager had stated to all staff, the Appellant wrote identifying the cause as modifications made to the document "Outpatients Department - Clinic contact Details" on 22 February 2024 and recommending a modification and review of that document. The reply at 4:30 pm, sent two hours after the conclusion of those stated office hours, stated that the task was being actioned and that the matter had been discussed with a named colleague that morning. The Respondent's amended List of Documents records no document created on 20 May 2024 recording that discussion, no communication sent by either person to the Service on that day, and the Respondent does not allege that the contact-details document was amended on or before 20 May 2024 or that the Switchboard staff were notified of the modifications made to it on 22 February 2024.

Particulars: paragraphs 89 to 110 of the notice to admit facts served 28 August 2026, admitted 8 September 2026.

S1.9 The office hours of the manager to whom the position reports. The position reports to the Switchboard Manager, and the role description names that person as the contact for the position. On 23 August 2023 she notified the Switchboard that she worked flex hours, that staff may have noticed she was starting and finishing at different times, and that her hours "would range from starting between 6-9am and finishing 2-5pm". The Respondent does not allege that fixed office hours were stated to the Switchboard staff at any time between 23 August 2023 and 17 May 2024. On 15 May 2024 at 1:15 pm the Appellant wrote asking that directives and changes be made in consultation with the team and asking that the office hours be shared so the department could be aware of the regular schedule, noting inconsistency in arrival and departure times. At 6:23 pm the Director replied, copied to the Manager and marked of High importance, stating that the email did not demonstrate the iCARE2 value of Respect and did not comply with the Code of Conduct, and asking the Appellant to retract it. At 7:09 pm the Appellant replied that requesting clarity on business hours was a reasonable question. On 17 May 2024 at 9:30 am the Manager wrote to the whole department stating that her hours could vary, that she would be in later between 0800 and 0830, that otherwise her hours were from 06:30 to 14:30, that she would not be in the office until around 11 am on Monday 20 May, and that moving forward she would send an email to switch to advise of any change to her office hours for the week. On 21 May the Director asked the Appellant whether there were one or more particular changes or directives about which he had concerns as to consultation and communication. On 18 June 2024 at 8:58 am the Manager wrote to Logan Switch stating that she was taking that day off and "I am sorry I haven't been there for you all over the past week".

Duty engaged: paragraphs 3 to 4 of that notice (the role description).

Particulars: paragraphs 70 to 88 of the notice to admit facts served 28 August 2026, admitted 8 September 2026.

S1.10 The Communication Book, 6 June 2023. A key responsibility of the position is to maintain discretion and exercise judgement where necessary to resolve problems within the scope of the role, in situations where precedence has not been set and procedures are not defined. A reminder concerning the monthly update of medical contact numbers was placed in the Communication Book and was removed. At 9:57 am on 6 June 2023 the Manager wrote to Logan Switch that the book was "not used to simply put your point across or make any indirect comments towards the team", and at 4:05 pm she wrote to the Director that she had taken the entry out the week before and that she felt the book was being used as a "burn book". The Respondent's pleading admits that pages were removed on or about 6 June 2023, and on 18 February 2026 the Respondent admitted that a page was removed and did not admit that it contained the Appellant's handwriting "because the respondent does not have a copy of the page". It also admitted that in an email to the Director of 6 June 2023 the Manager stated "I did raise my voice and asked him to please stop talking over the top of me". The Respondent does not identify the entry removed or its author, does not allege that the removed page has been located or that any copy exists, and does not list the Communication Book or any page or entry from it in its List of Documents.

Duty engaged: paragraphs 9 of that notice (the role description).

Particulars: paragraphs 143 to 153, 288, 292 of the notice to admit facts served 28 August 2026, admitted 8 September 2026. Paragraph 154 of that notice is not admitted.

S1.11 The matters raised in August and September 2023, and the response. On 7 August 2023 the Appellant raised matters concerning hours and workplace issues. The Manager acknowledged a rostering error and offered to roster him off the following day to give the required rest period, and the Director replied the same day. On 31 August 2023 he provided a written application headed "Request to Increase Working Hours to Full Time Rotational Roster", in which he expressed willingness to take on additional night shifts. On 4 September 2023 he confirmed in writing that he was able and willing to work any roster presented to him including the full 24-hour rotational schedule, and attached a draft roster spreadsheet. He was approved to commence full-time hours from 16 October 2023.

Particulars: paragraphs 26 to 38, 155 to 161 of the notice to admit facts served 28 August 2026, admitted 8 September 2026.

S1.12 The special pandemic leave application, February 2024 — the sequence. On 20 February 2024 the Appellant messaged the Switchboard Manager stating that he had tested positive for COVID-19 and asking what the policy was. The review decision records that she advised he was eligible for up to five days of paid leave under the Special Pandemic Leave request, that photo evidence of the test against his identification was required, and that she "requested that you send it to her so she could submit it in

MyHR". The Respondent pleads that she asked the Appellant to submit the request himself via myHR and attach a statutory declaration or photo evidence. The myHR submissions report records the initiator of that Smart Leave Request as the Appellant, and the Respondent does not allege that any other Switchboard employee was required to submit a Special Pandemic Leave request personally through myHR in February 2024.

The leave request history for Process Reference 15480560 records the Appellant creating a draft at 11:24:48 on 20 February 2024 and submitting the request at 11:41:27 the same morning; the request being declined at 12:23:43 on 21 February 2024; being submitted again at 06:07:32 on 28 February; being declined again at 09:06:07 on 29 February; being submitted a third time at 11:07:28 on 29 February; and being approved at 11:21:03 that morning, thirteen minutes and thirty-five seconds after that third submission. The history records the Appellant as having submitted the request on three occasions and the Manager as having declined it on two. The interval between the creation of the draft and the final approval recorded at 16:05:08 on 1 March 2024 is ten days, four hours, forty minutes and twenty seconds.

Two reasons for the declines appear on the record and they are not the same. The review decision records that on 21 February 2024 the Manager messaged that she had to decline the request because the minimum period for COVID leave was five days for a positive staff member, and that she noted the statutory declaration provided was sufficient evidence and asked him to resubmit his leave days; it records the Appellant as having messaged that he was confused about why his amended request was declined and that he had done everything as required. The Respondent pleads that she declined the application because on her assessment the required statutory declaration was not attached to the myHR submission, and pleads further that **a review indicates that in fact the attachments were present on the Appellant's submission**, and that this was human error on the background of high work demands.

Particulars: paragraphs 114 to 125, 132 to 142 of the notice to admit facts served 28 August 2026, admitted 8 September 2026.

S1.12A The authority to decide that application. The power to approve paid special pandemic leave was sub-delegated by an Instrument of Human Resource Sub-Delegation signed by the Health Service Chief Executive on 23 November 2022 and effective from 5 December 2022. That Instrument states that it "does not permit the further sub-delegation of the powers outlined in (a) above". In the leave request history the Switchboard Manager is identified as "Reviewer" and the Director as "Manager". The Respondent does not allege that either of them was a Band 9 delegate at any relevant time in February or March 2024, and does not identify the person who exercised the power sub-delegated by that Instrument in respect of Process Reference 15480560.

Particulars: paragraphs 126 to 131 of the notice to admit facts served 28 August 2026, admitted 8 September 2026.

S1.13 The complaint of 13 May 2024. On 13 May 2024 the Appellant made a complaint to the Metro South Health Ethical Standards Unit, which that Unit determined on 24 December 2024 constituted a public interest disclosure, reference 24-ESU-1130. **The content of the disclosure is not set out, and no connection between it and any subsequent conduct is pleaded or relied upon in this appeal.**

S1.14 Workplace representation. The Appellant notified an interest in the Switchboard delegate role, and the Respondent has admitted that the Union Encouragement Policy QH-POL-248 requires managers to take a positive, supportive role in relation to workplace delegates. He was endorsed as a workplace delegate on or about 3 November 2025. **No allegation of deliberate suppression is made.**

Particulars: paragraphs 178 to 179, 293 to 294 of the notice to admit facts served 28 August 2026, admitted 8 September 2026.

S1.15 What the employer states about its own systems and records. By letter of 5 June 2026 to the Commission, Metro South Hospital and Health Service stated that the requested fatigue risk management documents do not exist, that mandatory Fatigue Risk Management System training applies only to health practitioners and clinical assistants and that Switchboard staff are non-clinical, that the implementation of fatigue risk management assessment at the Switchboard occurred after 30 June 2024, that there have been no consequential changes to operating procedures over the period requested, and that all employee complaints relating to Switchboard operational errors are made directly to the line manager and "managed solely via email or verbally with the complainant". The Respondent does not allege that any fatigue risk assessment was conducted, that any fatigue risk management training was provided, that fatigue risk management assessment was implemented at the Switchboard, or that any change was made to the operating procedures of the Switchboard as a consequence of any employee complaint, at any time before 30 June 2024.

Duty engaged: paragraphs 12 of that notice (the role description).

Particulars: paragraphs 263 to 272 of the notice to admit facts served 28 August 2026, admitted 8 September 2026.

S1.16 Matters pleaded as context only. The matters at S1.10, S1.11, S1.12, S1.13 and S1.14 above, and at paragraph 3 below, are pleaded as part of the sequence of matters the Appellant raised and the responses he received. They are not relied upon as independent causative circumstances, and no retaliatory or reprisal connection between any of them and any other conduct is alleged. The causative circumstances relied upon in Stressor 1 are those at S1.1 to S1.9 and S1.15.

STRESSOR 2 — REMUNERATION

S2.1 The basis of the entitlement. The position is a continuous shift working role over the full 24-hour period, seven days a week, and the occupant must be able to work a roster covering multiple shifts over a 24/7 period. The entitlements pleaded below arise from that arrangement.

Duty engaged: paragraphs 2, 13 of that notice (the role description).

S2.2 Public holidays on which the Appellant was not required to work. Section 23 of the Hospital and Health Service General Employees (Queensland Health) Award provides that where an employee in receipt of the additional week's leave prescribed by clause 19.2(a) is rostered off on Easter Saturday, Easter Sunday, Show Day or Labour Day, that employee is to be paid an additional day's wage or granted a day's holiday in lieu. The Appellant was rostered off on public holidays between February and April 2024. As at 1 May 2024 payroll was still reviewing his entitlements regarding public holidays not required arising while he was a part-time employee, while the Manager confirmed that since his commencement of full-time employment all payments for public holidays not required had been processed.

Particulars: paragraphs 241, 255 to 256 of the notice to admit facts served 28 August 2026, admitted 8 September 2026.

S2.3 The correction sought, and where each direction sent it. On 8 April 2024 the Appellant asked the Manager to review his pay. On 9 April she directed him to raise the older dates through MyHR payroll enquiries. On 3 May 2024 Payroll wrote to the Manager, copying the Appellant, identifying four fortnights and asking her to "submit an AVAC to correct these shifts for each fortnight so Cory is paid corrected and his contracted hours are correct". On 10 May the Appellant wrote to Payroll. On 13 May 2024 Payroll replied to the Appellant: "I cannot see that any of the issues below have been corrected. Please speak to your Line Manager to have them corrected with an AVAC submitted through My HR". On 21 May the Manager wrote that she was still waiting for payroll confirmation. On 28 May she asked the Appellant to sign a validation of claims older than three months so that it could be escalated

for delegate approval.

Particulars: paragraphs 182 to 197 of the notice to admit facts served 28 August 2026, admitted 8 September 2026.

S2.4 Who could submit the correction. The myHR submissions report produced by Metro South Health records seven submissions for the Appellant in the period 1 February to 31 May 2024, of which five were Attendance Variation and Allowance Claims. It records the initiator of each of those five as the Manager, and the initiator of no such claim in that period as the Appellant.

Particulars: paragraphs 198 to 202 of the notice to admit facts served 28 August 2026, admitted 8 September 2026.

S2.5 The state of the record, and the Respondent's pleaded position. The report records the claim submitted on 28 May 2024, with an effective date of 30 March 2024 and a processing date of 30 May 2024, with a status of "Part Completed", and records the status of every other such claim in the period as "Completed". Neither of the two claim reference numbers identified by Payroll on 3 May 2024 appears in the report at all. The Respondent pleads that any discrepancies or errors were remedied in a timely manner and that there are no outstanding underpayments. On 18 February 2026 it admitted the Payroll instruction of 3 May 2024, the fact that the Manager submitted the claim on 28 May 2024, and the email of 21 May 2024.

Particulars: paragraphs 203 to 210, 296 to 298 of the notice to admit facts served 28 August 2026, admitted 8 September 2026.

S2.6 Rostering practices raised with Human Resources. On 20 May 2024 at 4:07 pm the Director wrote to LBH_HR following up a query she had raised in relation to an email received from a staff member about the department's rostering practices, asking that someone call her regarding those staff concerns, and attaching the Queensland Health Fatigue Risk Management Systems Implementation Guideline.

Particulars: paragraphs 222 to 223 of the notice to admit facts served 28 August 2026, admitted 8 September 2026.

STRESSOR 3 — THE ROSTER AND FATIGUE

S3.1 The rostering history, and the Appellant's proposals. A rostering error in August 2023 was acknowledged by the Manager, who offered to roster the Appellant off the following day to give the required rest period, and by the Director, whose email of 7 August 2023 the Respondent admitted on 18 February 2026 as stating that there had been a rostering error that was accidentally made. The Appellant applied in writing to increase his hours to a full-time rotational roster, confirmed he was able and willing to work any roster presented to him, expressed willingness to take additional night shifts, and provided a draft roster spreadsheet. The Director requested a document recording his rostered shifts over the preceding eight months.

Particulars: paragraphs 28, 30 to 31, 33, 35, 156 to 157, 159, 287 of the notice to admit facts served 28 August 2026, admitted 8 September 2026.

S3.2 The rostering errors acknowledged, and the decision withheld. On 26 April 2024 the Director wrote referring to a meeting on 16 April 2024 at which the Appellant had raised a roster line ending with night shifts followed by three days off and then returning to night shifts, and stated that the Manager was working to fix that error and would get in touch about alternative shifts. The Respondent does not allege that she did so. On 1 May 2024 the Appellant referred to section 4, clause 10.4.1 of the Operations Manual, which includes a toolkit required by management to manage fatigue, and stated that he had forwarded the toolkit to the Manager the previous week for review and action but had received no feedback; the Respondent does not allege that it was reviewed or that feedback was given before 18 June 2024. On 8 May the Director replied that she was following up with Human Resources. On 10 May 2024 at 2:08 pm she wrote to Human Resources forwarding his email, stating that she understood him to be raising concerns about how his manager was rostering and its impact on staff fatigue, that she and the Manager had run through the Roster Risk Assessment Matrix and put the rating at best at 11, moderate, that "I acknowledge there has been a few rostering errors made by Chloe with regards to Cory's line in past rosters", and that "Cory's roster will not be considered, however Cory is not yet aware of this as we cannot advise what is the roster we are planning on going with before we send out the implementation plan for this business case for change". On 21 May 2024 she asked the Appellant which particular changes or directives had concerned him.

Particulars: paragraphs 80, 211 to 221 of the notice to admit facts served 28 August 2026, admitted 8 September 2026.

S3.3 The Respondent's pleaded position on the roster. On 18 February 2026 the Respondent denied paragraphs of the Appellant's earlier notice on the stated grounds "because the comparator used does not represent a true comparator in the circumstances and because the roster was equitable". In November 2024 the employer released a Consultation Paper stating that the proposed roster "is intended to introduce a more equitable roster for Switchboard Services, Logan Hospital staff", and that it included "Redistribution of nights for greater equity based on FTE", a "Rotational roster for predictability and equity to all staff ensuring fair distribution of penalties", an additional eight-hour Saturday shift to assist with call volume, and the extension of a five-hour weekday shift to eight hours.

Particulars: paragraphs 171 to 175, 299 of the notice to admit facts served 28 August 2026, admitted 8 September 2026.

S3.4 The shifts of 17 and 18 March 2024. The Appellant was rostered to finish at 23:00 on 17 March 2024 and to commence at 06:00 on 18 March 2024, a break of seven hours. The shift commencing on 18 March fell on a Monday, the first shift of the working week. The Respondent does not allege that those consecutive shifts arose from a staff-initiated shift swap.

S3.4A The rest actually available between those shifts. The seven-hour figure is the interval between the rostered finish and the rostered commencement. It is not the rest that was available. The review decision records the Appellant's response of 9 August 2024 as stating that the break "did not include travel time", and: "If you factored in travel time, your break between shifts would have been less than 5 hours."

The shift of 17 March 2024 was a late shift, concluding at 23:00. **A late shift does not shorten the day that precedes it.** The Appellant had been awake throughout that day before commencing it, and the period of wakefulness is accordingly not measured from the commencement of the shift but from waking that morning. The Appellant will give evidence that from waking on the morning of 17 March 2024 to the conclusion of the shift at 23:00 was approximately sixteen hours; that he then travelled home, obtained approximately four hours of sleep, and travelled back to commence the first shift of the working week at 06:00 on 18 March 2024; and that the two shifts and the journeys between and about them required approximately four hours of travel in total. He will give evidence of where he lived and of the distance and travel time each way.

The Award and the employer's own fatigue policy are directed to the rest opportunity between shifts. The emergency duties pleaded at S1.7 and recorded at S3.7 below were performed on the shift of 18 March 2024 in that condition.

Particulars: paragraphs 251 of the notice to admit facts served 28 August 2026, admitted 8 September 2026.

Particulars: paragraphs 17 to 25, 224 to 227, 234 of the notice to admit facts served 28 August 2026, admitted 8 September 2026.

S3.5 The standard, as admitted. The review decision records that the employer's own response included an extract of the Award stating that employees must be provided with a break of not less than ten hours between the termination of one shift and the commencement of another, and that eight hours applied instead of ten only in specific circumstances. On 18 February 2026 the Respondent admitted that "The Employer's 'Fatigue Risk Management Policy' and the relevant Award require a minimum break of 10

hours between shifts, or 8 hours by written agreement", and admitted that the shifts of 17 and 18 March 2024 resulted in "a break of only 7 hours". It denied a further paragraph of that notice on the ground that in June 2020 the Appellant signed an agreement allowing an 8-hour break.

Particulars: paragraphs 257, 284 to 286 of the notice to admit facts served 28 August 2026, admitted 8 September 2026.

S3.6 The agreement of 17 June 2020, and its scope. By email of 7 July 2026 a Senior Consultant, Human Resources wrote that the 8-hour agreement signed on 17 June 2020 allows the Appellant to work with only an 8-hour break, "however this is only applied where staff initiated shift swaps have occurred". The Respondent pleads that in June 2020 the Appellant signed an agreement allowing an 8-hour break, and that the Appellant could refuse shifts at any time. The role description states, under mandatory requirements, that the position is a continuous shift working role and that the occupant must be able to work a roster which covers multiple shifts over a 24/7 period.

Duty engaged: paragraphs 13 of that notice (the role description).

Particulars: paragraphs 232 to 233 of the notice to admit facts served 28 August 2026, admitted 8 September 2026.

S3.7 The emergency workload carried on those shifts. The emergency workload carried on those shifts is recorded in the employer's Emergency Code Register. For 17 March 2024 the register records eight entries, including a MET call to Ward 2H Bed 7 at 19:16, a Code Blue to the same bed at 19:20 — four minutes later — and a further MET call to Ward 2Q Bed 5 at 21:16, each in the final hours of the shift before the 23:00 finish. For 18 March 2024 it records a Code Grey at the QAS triage airlock at 8:48, MET calls to Ward 6A Bed 8 at 9:20 and Ward 3DR Bed 14 at 11:31, a cancelled MET call to the same bed one minute later at 11:32, a neonatal MET call to Theatre 2 at 11:35 and a neonatal MET call to Birth Suites 5 at 13:09; six of the entries for that day bear times between 6:00 and 14:00, and so fall within the shift commenced at 06:00 after the rest pleaded at S3.4A. For 19 March 2024 it records a MET call to Ward 2I Bed 20 at 11:34 with the notation that it was called via switchboard. Those paragraphs were not admitted by the Respondent and will be proved by the register and any other admissible evidence.

Particulars: paragraphs 228 to 231 of the notice to admit facts served 28 August 2026.

S3.8 The leave of 19 March 2024. The Leave Takings Report produced by Metro South Health in respect of 19 March 2024 records the Leave Category as "Sick", the Leave Type as "Sick Leave", the Time Code as "SCK", the Leave Taken as 7.60 hours and the Leave Status as approved. The Respondent pleads that the Appellant took leave on that day but that it was paid leave, and that pursuant to clause 18.10 of the Award he is not entitled to fatigue leave because he was not performing overtime.

Particulars: paragraphs 235 to 237 of the notice to admit facts served 28 August 2026, admitted 8 September 2026.

S3.9 The enquiry of 8 April 2024, the delay, and the refusal of 1 May 2024. On 8 April 2024 the Appellant emailed the Manager requesting a review of his payment for the period 8 to 31 March 2024, noting the Award requirement as to a minimum ten-hour break. On 9 April she replied that she had escalated his enquiry regarding fatigue leave to Human Resources to confirm policies around this, acknowledged the importance of communication, and requested that he raise concerns with her as soon as they arose so she could action them sooner. On 24 April he emailed expressing dissatisfaction that more than two weeks had passed without response or payment. The Respondent does not allege that any response was made between 9 April and 1 May 2024, an interval of 23 days. On 1 May she responded that after a consultation with payroll and Human Resources the request for fatigue payment for 18 March 2024 would not be processed, on the basis of the existing 8-hour agreement, noting that he was able to terminate that agreement going forward. The review decision records that payroll was, as at that date, still reviewing his entitlements; that the employer's response did not mention the shift of 18 March 2024; and that there was uncertainty between the Appellant and the employer as to whether the 8-hour agreement continued to apply.

Particulars: paragraphs 242 to 250, 252 to 254 of the notice to admit facts served 28 August 2026, admitted 8 September 2026.

S3.10 The employer's own fatigue policy. The review decision records that the policy regarding fatigue leave following weekends and rostered days off is dated June 2020, and that the policy notes that many problems can be overcome regarding fatigue leave by either not rostering the employee who is on call for the first shift of the following day, or not rostering on call an officer who is rostered for the first shift on the following day.

Particulars: paragraphs 239 to 240 of the notice to admit facts served 28 August 2026, admitted 8 September 2026.

S3.11 The absence of fatigue risk management at the Switchboard. The matters pleaded at S1.15 above are repeated. The employer states that the fatigue risk management records do not exist, that the mandatory training applies only to health practitioners and clinical assistants, and that fatigue risk management assessment at the Switchboard was implemented only after 30 June 2024.

S3.12 The findings in the decision under appeal. The Respondent's own review decision of 24 October 2024 finds that the break between the shifts equated to seven hours; that even on the employer's own account the Appellant left a maximum of thirty minutes early and still did not receive a minimum eight-hour break; that the rostering of those two shifts amounted to unreasonable management action, given that it was in direct contradiction to the Award and the 8-hour agreement; that the Appellant sustained a personal injury of a psychological nature; and that his injury arose out of employment, to the extent that it arose out of factors 2, 3 and 4, where employment was a significant contributing factor.

Particulars: paragraphs 258 to 262 of the notice to admit facts served 28 August 2026, admitted 8 September 2026.

3. Subsequent conduct

(a) Meetings scheduled while the Appellant held current medical certificates. (b) Employment ended under abandonment provisions in October 2024 while certificates were current, and the Appellant was reinstated to his employment with an effective date of 20 September 2024, the reinstatement taking effect despite any payroll documentation. (c) The documents the Appellant provided to WorkCover Queensland in July and August 2024, and what the Respondent's amended List of Documents records of them.

Particulars: paragraphs 274 to 282 of the notice to admit facts served 28 August 2026, admitted 8 September 2026.

PART C — CONTENTIONS

1. ■ The onus, accepted. The Appellant accepts that he bears the onus of proving every element of his case on the balance of probabilities, including that his injury did not arise out of, or in the course of, reasonable management action taken in a reasonable way (Workers' Compensation Appeal Guide, Part 7.3; *Prizeman v Q-COMP* [2005] ICQ 43, where Hall P held that the onus is on the worker to establish that management action was unreasonable or taken in an unreasonable way, and that in the absence of evidence to support that assertion management action must be considered reasonable). The Appellant does not contend that the Respondent bears that onus, and the contrary contention in the Amended Form 9A filed 7 April 2026 is withdrawn.

2. How the onus is discharged. *Prizeman* presumes reasonableness only in the absence of evidence. The evidence is now on the record and is not in dispute. In particular the Respondent admits, as to its own documents and its own pleading: that access to the

database was removed and is not alleged to have been restored before 18 June 2024; that no document recording consultation with Switchboard operators before the change of 15 April 2024 is listed; that nine occasions of misdirected calls were reported by a clinician and nothing is alleged to have been sent to the Switchboard about them for six days; that no document of the discussion of 20 May 2024, no communication to the Integrated Respiratory Service, and no amendment to the contact-details document is listed; that the fatigue records do not exist, that the training applied only to health practitioners and clinical assistants, that fatigue assessment at the Switchboard began only after 30 June 2024, and that there were no consequential changes to operating procedures; and that complaints were managed solely by email or verbally with the complainant.

3. Compensable injury — section 32(1). The Appellant sustained a personal injury of a psychological nature arising out of, or in the course of, his employment, employment being **a significant contributing factor**. The injury is the diagnosed condition identified at Part A, arising from the course of conduct pleaded at Part B.2 and not from an event on a single day. The Respondent's own review decision so finds.

4. Section 32(5)(a), first limb — the matters at Stressor 1(a) and 1(h) are not management action. A directory the occupant of the position is required to keep accurate but cannot correct; emergency notifications reaching the wrong team; a clinical service unable to assist patients because a directory entry is wrong — these are the conditions in which the work was carried on, not action taken by the employer against the worker. One decision was taken, on 18 July 2023, for a stated project reason; what followed is a state of the system, and subsection (5) does not reach it.

5. Section 32(5)(a), second limb — where the matters pleaded are management action, it was not taken in a reasonable way. The change of 15 April 2024 was effective the day it was notified, with no record of prior consultation, against an instrument requiring a genuine opportunity to influence the outcome before a final decision. The remuneration correction was returned across four fortnights to the only person able to make it and is recorded as Part Completed. The fatigue enquiry drew no response for 23 days.

6. Delaney. Where multiple management actions form a course of conduct, their reasonableness may be evaluated in context and as a whole. That evaluation must nevertheless first identify the circumstances that constitute management action. Operational conditions that are not management action do not become management action merely because they occurred alongside management decisions.

7. Mahaffey. The presence of reasonable management action among the circumstances contributing to an injury does not necessarily enliven the exclusion where an unreasonable management action, or a circumstance that is not management action, also had a sufficient causal connection to the disorder. The rostering of the shifts on 17 and 18 March 2024 was found in the review decision to constitute unreasonable management action. The Appellant relies upon the medical and contemporaneous evidence to establish its contribution as part of the pleaded course of causative circumstances.

8. The effect of the admissions. The admissions are made for this proceeding under rule 49 and are relied upon as evidence of the existence and terms of the documents admitted, and of the matters the Respondent's statement of facts and contentions and List of Documents do not contain. The Appellant does not contend that any admission determines the reasonableness of any conduct; that is a matter for the Commission.

9. The hearing de novo. The Respondent says no finding in Review Decision 69983 binds the Commission. That is accepted, and it applies to the whole of the decision. The conclusion under appeal is a conclusion of that decision, reached upon the findings recorded in it.

10. The Respondent's pleaded case. Paragraph 27 of the amended statement of facts and contentions does not identify, by particular, date, document or cross-reference, the management action relied upon. No disciplinary process, formal performance management or communication described as a warning before 18 June 2024 is alleged.

Particulars: paragraphs 238, 295, 300 to 303 of the notice to admit facts served 28 August 2026, admitted 8 September 2026.

PART D — ORDERS SOUGHT

1. The appeal be allowed. 2. The decision of the Respondent dated 24 October 2024 be set aside. 3. It be declared that the Appellant sustained an injury within the meaning of section 32 of the Workers' Compensation and Rehabilitation Act 2003. 4. The Appellant's application for compensation be accepted. 5. Costs reserved.